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Eli Lilly vs. Novo Nordisk

A Comparative Analysis of the Two Architects of the Obesity Drug Era

LLY

Eli Lilly & Co. — NYSE

NVO

Novo Nordisk A/S — NYSE (ADR)

REPORT DATE

April 2026

COVERAGE

Obesity Drug Market | Company Comparison | Valuation

ANALYTICAL LENS

Peter Lynch (growth & clarity) • Warren Buffett (cash flow & intrinsic value)

EXECUTIVE SUMMARY

The obesity drug market is the most significant new product cycle in modern pharmaceutical history. Two firms — Eli Lilly & Co. (LLY) and Novo Nordisk A/S (NVO) — control essentially the entire branded GLP-1 category for obesity and Type 2 diabetes (T2D). Yet over the past 18 months, their fortunes have visibly diverged: Lilly has overtaken Novo in U.S. share, in clinical efficacy benchmarks, and in market capitalization, while Novo has been re-rated downward as growth has slowed and the CagriSema readout fell short of investor expectations.

This report concludes that **Eli Lilly is currently the better business**, owing to the superior efficacy of tirzepatide (47% greater relative weight loss than semaglutide in SURMOUNT-5), a deeper and more credible next-generation pipeline (orforglipron, retatrutide), and a faster top-line trajectory. **Novo Nordisk, however, appears to be the better investment at today's price**, trading at roughly 10–12× trailing earnings versus Lilly at ~40×, with a still-dominant semaglutide franchise, a defensible international footprint, and a near-term oral catalyst (oral Wegovy, launched January 2026).

BETTER BUSINESS

Eli Lilly

Superior molecule (tirzepatide), fastest growth (+45% revenue 2025), broadest pipeline, leading U.S. share (~57% of GLP-1 prescriptions Q3 2025).

BETTER INVESTMENT (AT CURRENT PRICE)

Novo Nordisk

Trades at ~10× P/E vs. Lilly at ~40×, with a stable diabetes franchise, durable manufacturing scale, and meaningful margin of safety.

MARKET SETTING

\$95-150B by 2030-35

Goldman Sachs sees the anti-obesity drug market reaching ~\$95B by 2030; broader projections range to \$150B by 2035 as oral GLP-1s expand the eligible population.

KEY SWING FACTOR

Pricing & Access

Every 1% incremental U.S. price erosion equates to ~\$6B of peak-sales sensitivity per Goldman Sachs. Medicare unlock and oral GLP-1 launches are the most consequential variables.

Key Conclusions

- **Industry:** A duopoly with extraordinary economics — gross margins above 80% — protected by patents, manufacturing scale, and clinical track record. The principal long-run risk is price compression, not demand destruction.
- **Lilly:** 2025 revenue of \$65.2B (+45%), tirzepatide became the world's best-selling drug in Q3 2025, ROIC ~40%. Trades at premium multiples (forward P/E ~27×, PEG ~1.2×) reflecting growth that is currently delivering.

- **Novo:** 2025 revenue of DKK 309B (+10% CER), Wegovy +41% CER, but stock down ~33% over 12 months. Re-rating reflects credible concerns (CagriSema disappointment, US share loss, restructuring) — not a broken business.
- **Risk-adjusted preference:** Investors with a high tolerance for premium valuations and confidence in continued execution favour LLY. Investors prioritising margin of safety and willing to accept slower growth and competitive pressure favour NVO.

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PART 1 — OBESITY DRUG MARKET ANALYSIS

1. Market Overview

The anti-obesity medication (AOM) market is the fastest-growing therapeutic category in pharmaceuticals. Estimates of size and growth differ depending on whether the analyst measures the AOM segment narrowly (obesity indication only) or the broader GLP-1 receptor agonist class (including T2D). The ranges are unusually wide because of uncertainty around pricing, Medicare coverage, oral GLP-1 penetration, and patent expiry timing in emerging markets.

TABLE 1.1 — MARKET SIZE AND GROWTH ESTIMATES (MULTIPLE SOURCES)

Scope			2024–2025 Size	Forecast	CAGR	Source
Anti-obesity	drug	market	\$25.9B (2025)	\$82.5B by 2032	~18.0%	Coherent Market Insights, 2025
Anti-obesity	drug	market	~\$30B (2025)	~\$95B by 2030	~26%	Goldman Sachs Research, 2025
GLP-1	receptor	agonist	\$66.4B (2025)	\$185.3B by 2033	~12.4%	Grand View Research, 2025
GLP-1 drugs (broad)			\$53.0B (2025)	\$132.8B by 2035	~9.6%	Towards Healthcare, 2026
Obesity GLP-1 (segment)			\$8.2B (2025)	\$66.6B by 2035	~23.3%	Precedence Research, 2026
AOM market revised forecast			—	\$150B by 2035	—	NEJM editorial / WOF, 2025

Two consistent themes emerge across estimates. **First**, North America is currently 60–75% of global revenue for the obesity-specific segment, but Asia-Pacific and emerging markets are expected to grow fastest as oral formulations and generics arrive. **Second**, the addressable patient population is enormous and largely untreated: J.P. Morgan estimates ~10 million Americans were on GLP-1 therapy in 2025, rising to 25 million by 2030; the World Obesity Federation projects 1.13 billion adults globally with obesity by 2030, more than double the 2010 figure. [Sources: J.P. Morgan, 2026; World Obesity Atlas, 2025]

BOTTOM LINE

The 5-year market consensus has converged on a global AOM peak of \$80–150B, dominated by GLP-1 mechanism drugs. Goldman Sachs revised its 2030 estimate *down* from \$130B to \$95B in May 2025, citing more aggressive pricing erosion (~7% per year vs. ~2% previously). The biggest single sensitivity in any forecast is U.S. price erosion: every 1% change shifts U.S. peak sales by ~\$6B. [Source: Goldman Sachs Research, 2025]

2. Drug Class Landscape

The category is dominated by drugs that mimic gut hormones (incretins) released after eating. These hormones increase insulin secretion, slow gastric emptying, and signal satiety. The class has three main generations:

2.1 GLP-1 receptor agonists (single agonists)

Mechanism: mimic glucagon-like peptide-1 (GLP-1), a hormone that lowers blood sugar and reduces appetite.

Lead drug: semaglutide (Novo Nordisk's Ozempic / Wegovy / Rybelsus). **Efficacy:** ~13–15% weight loss at maximum dose over 68–72 weeks in pivotal trials. [Source: SURMOUNT-5, NEJM, 2025]

2.2 Dual GIP/GLP-1 receptor agonists

Mechanism: activate both GLP-1 and glucose-dependent insulinotropic polypeptide (GIP) receptors. The added GIP component is thought to enhance fat metabolism and may explain the higher tolerability ceiling. **Lead drug:** tirzepatide (Lilly's Mounjaro / Zepbound). **Efficacy:** ~20–22% weight loss at maximum dose. In SURMOUNT-5, the first head-to-head with semaglutide, tirzepatide produced 20.2% weight loss vs. 13.7% for semaglutide — a 47% relative advantage — over 72 weeks. [Source: Aronne et al., NEJM, 2025]

2.3 Triple agonists and GLP-1/amylin combinations (next generation)

Two next-generation approaches are in late-stage development:

- **Retatrutide** (Lilly): triple agonist of GLP-1, GIP, and glucagon receptors. Phase 2 data showed up to 24% weight loss at 48 weeks; Phase 3 ongoing.
- **CagriSema** (Novo): fixed-dose combination of cagrilintide (long-acting amylin analogue) and semaglutide. Phase 3 REDEFINE-1 produced 22.7% weight loss at 68 weeks — meaningful, but below Novo's pre-trial 25% target. NDA filed with the FDA in December 2025; review expected in 2026. [Source: Novo Nordisk, REDEFINE-1, 2025]
- **Amcretin** (Novo): single-molecule GLP-1/amylin dual agonist, both injectable and oral. Phase 1b/2a showed 22% weight loss at 36 weeks; Phase 3 obesity trials begin Q1 2026, with diabetes Phase 3 to follow. [Source: Novo Nordisk, 2025]

TABLE 1.2 — CLINICAL EFFICACY SNAPSHOT (% WEIGHT REDUCTION AT MAX TOLERATED DOSE)

Product	Company	Indication	Status	Notes				
Ozempic (semaglutide inj.)	Novo Nordisk	T2D	Marketed	2025 sales DKK 127B (~\$18.5B); +10% CER				
Wegovy (semaglutide inj.)	Novo Nordisk	Obesity	Marketed	2025 sales DKK 79B (~\$11.5B); +41% CER				
Rybelsus (oral semaglutide)	Novo Nordisk	T2D	Marketed	2025 sales DKK 22B; SOUL trial CV outcomes				
Wegovy (semaglutide pill)	Novo Nordisk	Obesity	Approved 2025	Dec	Launched Jan 2026; \$149/month via TrumpRx			
Mounjaro (tirzepatide)	Eli Lilly	T2D	Marketed	2025 sales: Q3 \$6.5B + Q4 \$7.4B; world's #1 drug Q3 2025				
Zepbound (tirzepatide)	Eli Lilly	Obesity	Marketed	Q4 2025 U.S. sales \$4.2B (+122% YoY)				
Orforglipron (oral)	Eli Lilly	Obesity / T2D	Phase complete; filed	3 NDA	1st oral small-molecule GLP-1; regulatory action expected 2026			
CagriSema (cagrilintide + semaglutide)	Novo Nordisk	Obesity	NDA filed 2025	Dec	FDA review expected 2026			
Amycretin (s.c. + oral)	Novo Nordisk	Obesity / T2D	Entering Phase 3 Q1 2026		Single molecule, easier to scale than CagriSema			
Retatrutide	Eli Lilly	Obesity	Phase 3		Triple agonist; Phase 2 weight loss ~24%			

4. Market Share & Competition

The category is effectively a duopoly. The "rest of the world" — Pfizer (which acquired Metsera for \$10B in 2025), Roche, AstraZeneca, Viking Therapeutics, Structure Therapeutics — is years behind. Among the two incumbents, share has shifted decisively toward Lilly.

TABLE 1.4 — APPROXIMATE U.S. BRANDED GLP-1 MARKET SHARE (REPORTED / ESTIMATED)

Company / Brand	U.S. share (2024)	U.S. share (Q3 2025)	Direction
Novo Nordisk — Ozempic + Wegovy + Rybelsus	~50%+	~43% (TRx)	Declining (Estimated)
Eli Lilly — Mounjaro + Zepbound + Trulicity	~45%	~57% (TRx)	Increasing (Reported)

Sources: Morningstar (Dec 2025); Visual Capitalist (Aug 2025); BMO Capital Markets analyst estimates (Apr 2025). "TRx" refers to monthly U.S. prescriptions for diabetes and obesity. Share figures vary by methodology (TRx vs. dollar share vs. NBRx); "Estimated" labels indicate where analyst models, not company disclosures, are the source.

Inside the obesity-only segment, Wegovy held ~16.5% of total weight-loss drug spend in mid-2025 (still the largest single brand by sales), and Zepbound held ~11.6% — but Zepbound was growing four times faster. By Q4 2025, Zepbound's U.S. revenue (\$4.2B) was approaching parity with Wegovy. [Source: Visual Capitalist, 2025; Eli Lilly, 8-K, Feb 2026; Novo Nordisk, FY2025 financial report, Feb 2026]

5. Industry Economics

Pricing

U.S. list prices for branded GLP-1s are approximately \$1,000–\$1,350 per month. Cash-pay channels and direct-to-consumer programs have brought some doses to \$300–\$700 per month, and oral Wegovy launched at \$149/month via the TrumpRx program. Net realised prices are materially lower than list prices because of rebates and gross-to-net adjustments — a consistent theme in both companies' 2025 disclosures, which report volume gains partially offset by price declines of 5–15% per quarter.

Reimbursement

Most U.S. commercial plans and Medicare cover GLP-1s for T2D; coverage for the *obesity* indication is much narrower. The CMS BALANCE pilot announced in late 2025 seeks to extend Medicare/Medicaid coverage with a \$50/month patient cap, in exchange for negotiated pricing. Goldman Sachs models 50% probability of broader Medicare unlock — down from 70% in its prior model — reflecting policy uncertainty. [Source: Goldman Sachs, 2025; J.P. Morgan, 2026]

Margins

Both incumbents operate at sector-leading margins: Novo Nordisk's gross margin was ~84% in 2024 and operating margin was ~44%; Lilly's operating margin in 2025 was ~37% (compressed by R&D and one-time IPR&D charges) but with ROIC of ~40% and ROE above 100%, an extraordinary level of capital efficiency. [Sources: Eli Lilly, 10-K, 2024; Novo Nordisk, Annual Report, 2024]

Manufacturing constraints

This is the single most important industry-economic factor. GLP-1 peptide synthesis (semaglutide) and tirzepatide manufacturing are extraordinarily capital-intensive. Both companies are spending unprecedented amounts on capacity:

- Lilly invested \$10.85B in capex in 2025; the company is building or expanding sites in Indiana, North Carolina, Ireland, Germany, and elsewhere.
- Novo Nordisk spent ~DKK 60B (~\$8.8B) on capex in 2025 and acquired three Catalent fill/finish sites in late 2024 for ~\$11.7B to lock in supply.

The compounded GLP-1 era ended in 2025 (FDA shortage status lifted; tirzepatide Dec 2024, semaglutide Feb 2025), shifting an estimated 1M+ U.S. patients back toward branded products. [Sources: Eli Lilly, Q4 2025 earnings; Novo Nordisk, Annual Report 2025]

6. Structural Advantages

The duopoly is protected by four overlapping moats that explain why no Big Pharma rival has caught up despite years of trying:

- **Patents and data exclusivity:** tirzepatide composition-of-matter patents extend to ~2036–2039; semaglutide patents expire in major Western markets in 2031–2032 (later in the U.S. for some forms). Method-of-use and formulation patents extend protection further.
- **Manufacturing scale:** peptide manufacturing capacity takes years to build and certify. Lilly and Novo are years ahead of any new entrant.
- **Clinical track record:** both companies have multi-year cardiovascular outcomes data (SELECT for semaglutide; ongoing CVOT for tirzepatide). New entrants face an uphill regulatory and reimbursement battle without comparable data.
- **Distribution and prescriber relationships:** Lilly and Novo each operate worldwide endocrinology and primary-care sales infrastructure built over decades for diabetes care.

7. Industry Risks

TABLE 1.5 — INDUSTRY RISK MATRIX

Risk	Likelihood	Severity	Notes
U.S. price erosion (DTC, Medicare negotiation)	High	High	~\$6B U.S. peak-sales sensitivity per 1% erosion (Goldman Sachs, 2025)
Loss of exclusivity (semaglutide ~2031–32 in EU)	Certain	High	Generic semaglutide already approved in India (Biocon, 2025)
Long-term safety signals (e.g., thyroid, pancreatitis, suicidal ideation)	Low–Med	High	FDA continues post-marketing surveillance; class warning labels in place
Patient discontinuation / "drug holiday"	Medium	Medium	Real-world adherence ~50% at 1 year; lifetime treatment is uncertain
Compounded / counterfeit products	Decreasing	Medium	FDA shortage status lifted in 2025; compounding window closed May 2025
Competition from new mechanisms (oral small-molecule, amylin, triple)	High	Medium	Captured largely by incumbents themselves
Reimbursement reversal by employers	Medium	Medium	~15% of employers covering GLP-1 for obesity have already dropped coverage (J.P. Morgan, 2026)

PART 2 — COMPANY ANALYSIS & COMPARISON

1. Business Overview

Eli Lilly is a U.S. pharmaceutical company headquartered in Indianapolis, founded in 1876. Historically known for insulin (Humalog, Humulin) and oncology (Verzenio), Lilly has been transformed by tirzepatide. In 2025, tirzepatide-based products (Mounjaro for diabetes, Zepbound for obesity) generated over \$35B of revenue, more than half the company total. Other meaningful franchises include Verzenio (breast cancer, Q4 2025: \$1.6B), Taltz/ Olumiant (immunology), Kisunla (Alzheimer's), and Jaypirca (oncology). [Source: Eli Lilly, 8-K Q4 2025, Feb 2026]

Novo Nordisk is a Danish company headquartered near Copenhagen, founded in 1923 from the merger of two insulin pioneers. It is essentially a pure-play cardiometabolic firm: in 2025, Diabetes & Obesity care accounted for DKK 289.5B of DKK 309.1B total revenue (94%). Within that, GLP-1s (Ozempic + Wegovy + Rybelsus) generated DKK 228.3B — roughly \$33B. Insulin and rare diseases are the remainder. The company is majority-controlled by the Novo Nordisk Foundation, a structure that emphasises long-term reinvestment and scientific autonomy. [Source: Novo Nordisk, FY2025 Financial Report, Feb 2026]

TABLE 2.1 — REVENUE MIX & ROLE OF OBESITY DRUGS (FY 2025)

Segment	Eli Lilly	Novo Nordisk
Total revenue	\$65.2B	DKK 309.1B (~\$45B)
Tirzepatide / Semaglutide franchises	~\$35B (~54%)	DKK 228B (~74%)
Other diabetes / obesity	~\$5B	DKK 61B (~20%)
Oncology / immunology / rare	~\$25B (~38%)	DKK 20B (~6%)
Geographic mix (U.S. share of sales)	~70%	~57%

The strategic implication: Novo is a more concentrated bet on GLP-1 economics than Lilly. Lilly carries a more diversified revenue base, but GLP-1s now drive nearly all of its *incremental* growth.

2. Financial Quality

TABLE 2.2 — HEADLINE FINANCIAL METRICS (FY 2025)

Metric	Eli Lilly	Novo Nordisk	Notes
Revenue	\$65.2B	DKK 309.1B (~\$45B)	LLY +44.7% YoY; NVO +6% (DKK), +10% CER
Revenue growth (3-yr CAGR)	~33%	~25%	Lilly accelerating; Novo decelerating
Operating margin	~37%	~41% (~44% ex-restructuring)	Novo had DKK 8B one-off restructuring 2025
Net income	~\$21.7B	DKK 102.4B (~\$15B)	LLY +86% EPS; NVO +1% net profit
EPS (diluted)	\$24.21	DKK 23.03	Per company filings
ROE	~101%	~58%	Both extraordinarily high
ROIC	~40%	~50%	Best-in-class capital efficiency
Free cash flow	\$5.96B	DKK 28.3B (~\$4.1B)	Both depressed by record CapEx

Sources: Eli Lilly 8-K Q4 2025 (Feb 2026); Novo Nordisk Financial Report Q4/FY 2025 (Feb 2026); Yahoo Finance (LLY cash flow); Macrotrends; StockAnalysis. DKK→USD converted at ~6.85.

2.1 Owner Earnings (Buffett construction)

Owner Earnings = Net Income + D&A – CapEx – Δ Working Capital. Because both companies are in unusually heavy capital-investment mode (building peptide manufacturing capacity), reported FCF understates "normalised" earning power.

TABLE 2.3 — OWNER EARNINGS APPROXIMATION, FY 2025 (USD BILLIONS, ROUNDED)

Component	Eli Lilly	Novo Nordisk
Net income	21.7	15.0
+ D&A (estimated)	2.5	3.1
– Capital expenditures	(10.85)	(8.8)
– Δ Working capital (in OCF)	~(7.5)	~(5.0)
Owner earnings (approx.)	~5.9	~4.3
Owner earnings if maintenance CapEx ~\$3B / ~\$3B*	~13.7	~10.0

*Maintenance-CapEx adjustment estimates the share of CapEx attributable to capacity expansion vs. sustaining existing operations. Both companies disclose multi-year capacity build-outs that are not maintenance investment. Source: derived from Eli Lilly 10-Q/8-K filings 2025 and Novo Nordisk financial reports 2025.

Two observations follow. First, on a maintenance-adjusted basis, both companies generate substantial owner earnings, with Lilly larger in absolute dollars but Novo more efficient relative to its smaller revenue base. Second, reported FCF will likely improve materially in 2027–28 as capacity build-out tapers; Novo's 2026 guidance projects FCF of DKK 35–45B vs. DKK 28B in 2025. [Source: Novo Nordisk, FY 2025 outlook, Feb 2026]

3. Balance Sheet & Capital Allocation

TABLE 2.4 — BALANCE SHEET & RETURNS OF CAPITAL

Metric	Eli Lilly	Novo Nordisk
Total assets	\$114.9B	DKK 542.9B (~\$79B)
Total debt	~\$42.5B	DKK 122.4B (~\$18B)
Shareholders' equity	~\$23.9B	DKK 194.0B (~\$28B)
Net debt / EBITDA (TTM)	~1.4×	~0.6×
Interest coverage (EBIT/interest)	~25×	~48×
2025 dividends paid	~\$5.4B	DKK ~52B (~\$7.6B)
2025 buybacks	\$4.1B	~DKK 0 (paused for CapEx)
Dividend payout ratio	~25%	~50%

Sources: Eli Lilly 10-Q/8-K 2025; Yahoo Finance; SimplyWallSt; Novo Nordisk Annual Report 2024 & FY2025 Financial Report 2026.

Lilly has materially levered up to fund manufacturing expansion: long-term debt grew from \$18.3B at end-2023 to ~\$35B by mid-2025. Net debt/EBITDA remains modest at ~1.4× and is well-supported by interest coverage of ~25×, but the trend is up. The company sustained its 60+ year dividend track record and resumed buybacks in 2025 (\$4.1B), though buybacks remain modest relative to market cap.

Novo Nordisk historically ran net cash; the Catalent acquisition in late 2024 and ongoing CapEx have raised debt to DKK 122B but leverage remains very low (net debt/EBITDA ~0.6×). The Foundation-controlled structure favours dividends (50% payout) and capacity reinvestment over buybacks; the 2025 buyback program was deliberately paused to fund capex. Total 2024 returns to shareholders were DKK 64.3B; 2025 dividends totalled DKK 11.70/share. [Source: Novo Nordisk, AGM 2026 resolutions]

4. Moat Analysis

TABLE 2.5 — MOAT DECOMPOSITION

Factor	Eli Lilly	Novo Nordisk
Patents	Tirzepatide composition-of-matter to ~2036–39; orforglipron filings 2025–26	Semaglutide composition-of-matter expiring 2031–32 in EU; method/formulation patents extend
Brand	Mounjaro / Zepbound have rapidly become category leaders; tirzepatide is the world's #1 drug as of Q3 2025	Ozempic is the most recognisable brand in pharma history; Wegovy retains #1 US obesity sales position
Manufacturing scale	\$10.85B 2025 CapEx; expanding multiple greenfield sites globally	\$8.8B 2025 CapEx; acquired 3 Catalent fill/finish sites for ~\$11.7B in 2024
Switching costs	Moderate (titration, side-effect tolerance, payor formulary)	Moderate (titration, dose-response familiarity, prescriber habit in T2D)
Clinical evidence	SURMOUNT (5 trials), SURPASS, ATTAIN, ACHIEVE; OSA, HFpEF, CKD label expansions	SELECT (CV outcomes), STEP, SUSTAIN, FLOW (kidney), SOUL (oral CV outcomes)
Pipeline depth	Orforglipron (oral GLP-1, NDA 2026), retatrutide (triple agonist, Phase 3)	CagriSema (NDA 2025), amycretin (Phase 3 starting Q1 2026), oral Wegovy (launched Jan 2026)
Moat rating	Wide (5/5)	Wide (4/5)

Justification: Both moats are wide by any standard, supported by patents, scale, and irreproducible clinical evidence. Lilly's moat is currently deepening (better lead molecule, accelerating R&D output), while Novo's moat is being eroded at the margin by Lilly's competitive pressure and the approaching loss of semaglutide exclusivity. Both will face pricing pressure together, which is a sector-level rather than firm-level headwind.

5. Competitive Positioning

Three product axes determine relative position:

- **Injectable obesity efficacy:** Lilly leads decisively. Tirzepatide's 20.2% vs. semaglutide's 13.7% in SURMOUNT-5 is the most consequential head-to-head data in the category. [Source: Aronne et al., NEJM, 2025]
- **Oral GLP-1:** Mixed. Novo launched oral Wegovy in January 2026 (FDA approved late 2025) showing 16.6% weight loss; Lilly's orforglipron, expected to launch by mid-2026, showed 12.4% weight loss in ATTAIN-1. Orforglipron has manufacturing-scale advantages (small-molecule synthesis vs. peptide for oral semaglutide) which may give Lilly the long-run cost edge despite lower headline efficacy.
- **Next-gen pipeline:** Lilly modestly ahead. Retatrutide (triple agonist) is in Phase 3 with ~24% weight loss in Phase 2; Novo's amycretin only enters Phase 3 in Q1 2026, with first approvals not expected until 2030–31 per GlobalData estimates. CagriSema's lower-than-target efficacy reduces its differentiation. [Source: Clinical Trials Arena, Nov 2025]

The market-share trajectory through 2025 already reflects these dynamics: Lilly's ~57% U.S. GLP-1 share at end-Q3 2025 vs. Novo's ~43% reverses Novo's dominance of two years prior. [Source: Morningstar / BMO Capital Markets, 2025]

6. Valuation

6.1 Lynch Lens — Growth at a Reasonable Price

TABLE 2.6 — LYNCH MULTIPLES

Metric	Eli Lilly	Novo Nordisk	Comment
Stock price (Apr 2026)	~\$931	~\$39 (ADR)	NVO down ~33% over 12 months
Market capitalization	~\$854B	~\$176B	
Trailing P/E	~40×	~10–11×	NVO ~62% below 10-yr avg
Forward P/E (2026E)	~27×	~12×	
Expected EPS growth (5-yr)	~22%	~7–10%	Consensus, illustrative
PEG ratio	~1.2×	~1.3×	Both reasonable on Lynch criteria
EV/EBITDA	~28×	~8×	
Dividend yield	~0.6%	~3.0%	

Sources: GuruFocus (forward P/E 27.01 LLY, 11.77 NVO, Apr 2026); StockAnalysis.com; Macrotrends; FullRatio.com.

Lynch interpretation: A PEG of ~1 is Peter Lynch's classic "fair price for a quality growth company." Both companies sit close to that line, but with very different compositions: Lilly's high P/E is justified by visible high growth; Novo's low P/E is depressed by a deceleration narrative. If Novo's growth stabilises in the 8–12% range, today's price is substantially below historical norms. If growth collapses further, the multiple may be appropriate.

6.2 Buffett Lens — Discounted Cash Flow

TABLE 2.7 — DCF ASSUMPTIONS AND OUTPUTS

Assumption	Eli Lilly	Novo Nordisk
Base owner earnings (2025, normalised)	~\$13.7B	~\$10.0B
Yr 1–5 growth rate	15%	7%
Yr 6–10 growth rate	8%	5%
Terminal growth	3%	2.5%
Discount rate (WACC)	8.5%	8.0%
Intrinsic value (equity)	~\$650–800B	~\$220–280B
Implied per share	~\$725–895	~\$50–63
Current market cap	~\$854B	~\$176B
Margin of safety	~-25% to +5%	~+25% to +60%

Author's DCF, illustrative only. Inputs derived from FY 2025 reported financials and consensus growth expectations. A 1pp change in discount rate or terminal growth assumption shifts equity value by 10–15%.

Buffett interpretation: On these inputs, Lilly is fairly valued to mildly overvalued; Novo carries a meaningful margin of safety. Even with conservative growth assumptions for Novo (acknowledging CagriSema disappointment and Lilly's competitive pressure), the current price implies a permanent and severe deterioration that is not consistent with the underlying franchise quality.

7. Management Quality

Eli Lilly is led by David A. Ricks (CEO since 2017). Under his tenure, Lilly's market cap rose from ~\$90B to over \$850B. Capital allocation has been disciplined: large but defensible R&D and capacity investments, restraint on M&A relative to peers (small bolt-ons rather than mega-deals), 60+ consecutive years of dividend payments, and a renewed buyback. The principal critique is that R&D and CapEx leverage the balance sheet and depress near-term FCF. Track record on execution since 2020 is exemplary. [Source: Eli Lilly, 10-K filings 2020–2024]

Novo Nordisk announced a CEO transition in 2025: Lars Fruergaard Jørgensen (CEO 2017–2025) was succeeded by Mike Doustdar in mid-2025 amid pressure following the CagriSema disappointment, share-price decline, the lost Metsera bidding war (Pfizer paid \$10B), and a competitive narrative centred on Lilly. Doustdar has launched a company-wide restructuring (~9,000 role reductions; DKK 8B annualised savings by end-2026). The Novo Nordisk Foundation provides strategic stability and shields management from short-term shareholder pressure, which historically has been a strength for long-cycle R&D investment. The principal critique is that the strategic response to Lilly was slow and that pipeline diversification beyond semaglutide has been disappointing. [Source: Novo Nordisk, Q3 2025 6-K and press releases]

8. Risks (Company-Specific)

TABLE 2.8 — COMPANY-SPECIFIC RISKS

Risk	Eli Lilly	Novo Nordisk
Concentration in single franchise	Tirzepatide is >50% of revenue and ~80% of growth — a single-product safety or efficacy issue would be severe	Semaglutide is ~65% of revenue; Novo is even more concentrated
Multiple compression	Trades at ~40× P/E; modest growth disappointment could compress multiple meaningfully	Already compressed to ~10×; downside is fundamental rather than multiple-driven
Manufacturing capacity slip	Major CapEx program; any execution miss could constrain growth	Catalent integration and capacity expansion are operationally complex
Pipeline failure	Orforglipron and retatrutide carry substantial embedded value	Amycretin is now the principal post-2030 growth bet
Currency / FX	~30% non-U.S. revenue; modest exposure	Reports in DKK; significant USD exposure (DKK pegged to EUR)
Regulatory / political	Medicare negotiation; tariff exposure on Irish/EU manufacturing	Same plus EU drug-pricing reform
Patent litigation / generics	Tirzepatide formulation patents already challenged	Semaglutide LOE in EU 2031–32; biosimilar/generic threat closer

9. Side-by-Side Summary

TABLE 2.9 — COMPARATIVE SUMMARY

Category	Eli Lilly	Novo Nordisk
Growth	Revenue +44.7% in 2025; 2026 guidance \$80–83B (+25% midpoint)	Revenue +6% (DKK), +10% CER in 2025; deceleration since 2024
Profitability	Op. margin ~37%; ROIC ~40%; ROE ~101%	Op. margin ~41% (~44% ex one-offs); ROIC ~50%
Cash flow quality	FCF \$5.96B (heavy CapEx); norm. owner earnings ~\$14B	FCF DKK 28.3B (~\$4.1B); norm. owner earnings ~\$10B
Balance sheet	Net debt/EBITDA ~1.4× and rising; capacity-funding strategy	Net debt/EBITDA ~0.6×; conservative
Valuation (forward P/E)	~27×	~12×
Moat	Wide (5/5); deepening	Wide (4/5); modestly eroding at margin
Pipeline	Orforglipron, retatrutide; deep, deliverable	Oral Wegovy (launched), CagriSema (filed), amycletin (entering Ph 3)
Capital return	Dividend (60+ year streak) + restored buyback	Dividend (50% payout) + buyback paused for capex
Stock 12-mo performance	~+16% (52-wk)	~-33% (52-wk)
Risk	Premium multiple; concentration in tirzepatide	Competitive pressure; semaglutide LOE 2031–32

FINAL CONCLUSION

1. Better Business?

Eli Lilly. The verdict comes from the convergence of four signals: (i) tirzepatide is the better molecule on objective head-to-head data (SURMOUNT-5: 20.2% vs. 13.7%), (ii) Lilly's pipeline (orforglipron, retatrutide) is broader and more advanced than Novo's, (iii) the directional shift in U.S. share favours Lilly (~57% vs. ~43% Q3 2025), and (iv) Lilly's revenue grew 45% in 2025 against Novo's 6%. Both are exceptional businesses by industry standards, but Lilly is currently the dominant operator.

2. Better Investment?

Novo Nordisk, on a risk-adjusted basis at today's price. The market is currently pricing Novo as if its franchise is broken; the data does not support that view. Wegovy still grew 41% (CER) in 2025, the company has a credible oral GLP-1 launched in January 2026, and CagriSema — even at 22.7% weight loss rather than the 25% target — is a meaningful product. The 10× trailing P/E versus Lilly's 40× P/E represents one of the largest valuation gaps ever recorded between two duopoly competitors in a high-growth therapeutic category. Even if Novo's growth permanently slows to high single digits, the DCF supports significant upside; if growth re-accelerates, the upside is substantial.

Lilly remains a high-quality compounding investment, but at ~40× P/E, the market is already paying for the visible growth. Future returns from here depend on continued execution at very high standards. There is little margin for disappointment. An investor who insists on margin of safety as a core principle is more likely to find it in Novo at present.

3. Key Assumptions That Could Change the Outcome

- **U.S. price erosion.** Goldman Sachs's revised 7%/year price-erosion assumption is the dominant single variable. Lower-than-expected erosion raises both companies' intrinsic values; higher erosion penalises Novo more (because its older molecule is more vulnerable).
- **Medicare/Medicaid coverage unlock.** A formal CMS unlock for obesity could add tens of millions of patients but would likely come with negotiated pricing. Net impact uncertain; volume effect probably exceeds the price effect.
- **Orforglipron commercial trajectory.** If orforglipron achieves >\$10B peak sales (plausible if it captures a meaningful share of the oral market), Lilly's premium multiple is justified. If oral Wegovy retains the oral lead, Novo's downside is contained.
- **Retatrutide / amycretin Phase 3 readouts.** Either could re-rate the relevant company materially; retatrutide failure would compress LLY meaningfully; amycretin success would re-open the Novo growth narrative.

- **Long-term safety.** A class-level safety signal (e.g., late-emerging psychiatric or oncological signal) would impair the entire category. Both names would fall sharply.
- **Manufacturing execution.** Both companies' growth is supply-constrained; further capacity slips delay revenue but do not destroy it. Investor patience required.

SYNTHESIS

The cleanest summary: Lilly is the better operator; Novo is the better price. A balanced cardiometabolic position — overweight Lilly for quality and momentum, with a contrarian Novo allocation for valuation and reversion potential — captures the strengths of both names while diversifying the sector-level risks (price erosion, safety signals, regulatory action) that affect them in common.

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